Teacher and cover planner

Mental health and support

Wednesday 20 January 2027 · P3 · 40 minutes · Year A

GROW · up to five learners · Spring 1 Week 3 · formative

Theme Resilience & Change. Thread tags: SMSC S/So; British Values IL/MR.

Exact outcome

Understand mental health and where to get support.

Source GROW Weekly Spring row 66, C66 verified. Owner description is the lesson focus. No awarded unit or accreditation is claimed.

Prepare

Books, writing tools, usual aids and printed Support Cards. Choose School or Public slides and planner. Shared resources suit both. No internet, real account, personal story, symptom list or named service is needed. Agree the private pause and help route before starting. Evidence Sheet page 2 offers both main tasks as an editable typed route.

Forty minute sequence

Phase	Time	Teaching
Arrival and starter	0–5	Settle, agree private help and recall that everyone has mental health.
Input	5–10	Explain mental health and possible signs. Support can be asked for at any time.
Model and guided work	10–17	Read A. Model clue, support type, opener and fallback. Match or improve advice.
Private hinge and choice	17–22	Check signs do not prove a diagnosis. Choose task and response mode.
Individual task	22–34	Support map B or advice note C. Optional private one-line card.
Review	34–38	Check against success list; one next step and pupil response.
Private exit	38–40	Explain everyone has mental health or correct a myth about help.

Processing time and brief quiet pauses sit within these forty minutes. Reduce writing before rushing. Private card and exit may use a fictional response or pass.

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Teaching and task choices

Input

Everyone has mental health. It is part of health. It links to thoughts, feelings and how we cope with daily life. Mental health can change. Having mental health does not mean having an illness. A person may need support if they feel low or worried for a while, sleep very differently, stop doing things they like or find daily tasks hard. Signs can have many causes. One sign cannot tell us a diagnosis. A person can ask for help at any time, even with no clear sign.

Model A

A young person has a new morning routine. For several days they feel worried and find it hard to focus on a short task. They want help to plan the first step. Model: Worry and focus are making a daily task harder. School staff could listen and help plan one clear first step. They can help find more support if needed. The person could ask: Can we talk in private? I need help with the new routine.

Guided work and fade

Choice 1: match School staff to help with the routine. Choice 2: improve “Wait until it is severe before asking.” Model a private opener, then reduce answer cues. Keep the model and usual access aids. A need for help does not require a diagnosis.

Two equivalent main tasks

Choice 1 Support map for B: explain mental health; note a possible clue; choose a support type and reason; give a private opener and fallback. Choice 2 Advice note for C includes the same five elements. Either can be in a book, typed, aided or exactly scribed. No public presentation or compulsory role play.

Supported

Choose either task. Read one short part at a time. Offer two support types if needed. Keep bank, starter and model. Point, type, use an aid or an exact scribe. Record content cues apart from usual access.

Independent

Choose either task. Explain mental health, identify a clue, choose a fitting support type with a reason, and give an opener and fallback. Keep the model and normal aids.

Aspire

Choose either task. Explain why a sign is not a diagnosis. Compare two support types and adapt the help route if the first adult is unavailable. No personal account or public speaking.

TA focus

I do: model A without asking for pupil stories. We do: offer the two guided choices. Independent: check each pupil privately, record aids and answer cues, and keep usual access. Do not fill missing answers or press a pass.

One line to keep

Offer the private optional card in the final two minutes of individual work. “If I needed support I could…” may be finished privately by type only. A fictional line, blank or pass is valid. The card is not assessed or collected.

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Aids for each task

Every task keeps its words, starter and visible model. The same seven aid sets appear in Support Cards. Learners can plan in any language.

Starter

Words: health · thoughts · feelings · everyone

Start: Mental health is…

Model: Mental health is part of everyone's health.

Signs and meaning

Words: may · change · daily life · help

Start: The story says… This may mean…

Model: The story says worry makes a task hard. Support may help.

Model and guided choice

Words: listen · school staff · first step

Start: The person could ask…

Model: The person could ask school staff for help with the new routine.

Private hinge

Words: sign · cause · diagnosis · support

Start: A sign can… It cannot…

Model: A sign can show a need for support. It cannot prove a diagnosis.

Main task choice

Words: clue · adult · request · reason

Start: The person could… because…

Model: The person could ask a trusted adult because daily life feels hard.

Private keep card

Words: if · support · could · ask

Start: If I needed support I could…

Model: If the story person needed support they could ask school staff in private.

Review and exit

Words: check · next step · help · reply

Start: One safe next step is…

Model: One safe next step is to ask another trusted adult if help is not enough.

Prompt ladder

Point to the task. Ask a short private question. Rephrase. Offer a choice or scaffold. Model a first step if needed, then return the task. Allow time. Log content cues apart from normal reading, typing, communication or scribing access.

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Checks and planned access

Success criteria

Explain that everyone has mental health.

Use a fictional clue to explain why support may help, without a diagnosis.

Choose a support type, a private opener and a next route if more help is needed.

Main private hinge

A person has a change in sleep and asks for help. Which fits? A A sign proves a diagnosis. B A sign may mean support would help. C Only a doctor has mental health. D Wait until the problem is severe. Answer B. A over-diagnoses; C confuses universal health with illness; D delays help. Ask privately by point, aid, note or words. Pass is unobserved, not an error.

Parallel and decision

The story person feels low and would like help. A Ask a trusted adult in private. B Keep it secret until it gets severe. C Tell the whole group. D Diagnose them from one sign. Answer A. If the first hinge is wrong or unclear, re-model sign versus diagnosis and help at any time, then use the parallel. If still unclear, keep supported practice and record the gap.

Optional fictional checks

F Who could the story person ask? M Why might the clue suggest support? H Why can the class not diagnose from it? Never use cold calling or ask for real symptoms.

Cognition and learning

Read one short part at a time. Highlight the given clue. Keep examples and two support choices available. Use a written sequence.

Communication and interaction

Allow processing time, short private choices, a communication aid, typing or exact scribing with quiet readback and correction. Do not assess public speech.

Regulation sensory and language

Use predictable stages, neutral task, pass and supervised pause. Keep usual physical/sensory aids and comfortable position. Teach terms explicitly, read together and plan in any language. Words carry meaning without colour cues.

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Evidence cover and private support

Evidence

Collect only the fictional map or advice note. Check universal mental health, possible clue without diagnosis, appropriate support and reason, opener and fallback. Record actual aids, content prompts, one NS and reply. Partial work is retained. Do not infer a pupil’s health, diagnosis or real help-seeking from a story response. Private card and real concerns stay out of the learning record.

Self assessment

Learning check only: Ready to explain / With a prompt / Need another example. It is not a mood, symptom or mental-health score. All boxes begin blank.

Feedback and portfolio

Keep the first response. Give one specific NS. The pupil replies in their own colour or usual mode. E records actual saved work. R follows only when Voice and Audience are present with one NS and the pupil's actual response. Use work codes, never names. Offer to the Communication portfolio; staff confirm any mapping.

Safeguarding

Any disclosure goes to the safeguarding lead through school procedure. Listen calmly, reassure and explain that staff must share a concern with people who can help. Do not probe, investigate or promise secrecy. Use the school's immediate-concern route when needed. Keep real details in the school's safeguarding record, separate from learning evidence. A pupil unable to identify a trusted adult is supported privately, not pressed.

Discreet step out

Before the lesson agree a discreet HELP or PAUSE signal. Staff acknowledge it quietly and use the familiar supervised place and agreed adult support. No public explanation is needed. Offer a neutral task, private catch-up and supported return. Do not send a pupil away alone.

Private card

The one-line personal card is optional and stays with the pupil. Do not collect, photograph, display, compare or copy it into evidence. Any space may stay blank. A fictional response or pass is valid. No adult name, health detail or real story is required.

Non specialist cover

Read the supplied definitions and original cases only. Use a generic support type rather than improvise service details. No live search or personal check-in. If time is short, model A, check privately, use one main task and give one NS/reply. Keep private help available. Do not bank missing evidence.

Where this leads

This helps me ask for support when learning, joining a group or doing future work.

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